

PACKAGE INSERT – PRIACIN

1. Trade Name of the Medicinal Product

PRIACIN

2. Qualitative and Quantitative Composition

Each PRIACIN film-coated tablet 20mg, contains 20 mg of simvastatin.

Each PRIACIN film-coated tablet 40mg, contains 40 mg of simvastatin.

For excipients see 6.1.

3. Pharmaceutical Form

PRIACIN 20mg: Peach, round, convex, film coated tablets with diameter of nucleus 8mm

PRIACIN 40mg: Pink, round, convex, film coated tablets with diameter of nucleus 10.3mm

Film-coated tablets for oral administration.

4. Clinical Particulars

4.1. Therapeutic Indications

Reductions in risk of CHD mortality and cardiovascular events

PRIACIN is indicated in patients at risk of coronary events because of existing coronary heart disease, diabetes, peripheral vessel disease, history of stroke or other cerebrovascular disease,:

- reduce the risk of total mortality by reducing the CHD deaths
- reduce the risk of non-fatal myocardial infarction and stroke
- reduce the need for coronary and non-coronary revascularization procedures

Hyperlipemia

PRIACIN is indicated as an adjunct to diet to reduce elevated total-C, LDL-C, Apo B, and TG, and to increase HDL-C in patients with primary hypercholesterolemia, heterozygous familial hypercholesterolemia or combined (mixed) hyperlipidemia when response to diet and other non-pharmacological measures is inadequate. PRIACIN therefore lowers the LDL-C/HDL-C and the total – C/HDL-C ratios.

Paediatric patients with heterozygous familial hypercholesterolemia

PRIACIN is indicated as an adjunct to diet to reduce total-C, LDL-C, Apo B, and TG levels in adolescent boys and girls who are at least one year post-menarche, 10-17 years of age, with heterozygous familial hypercholesterolemia (HeFH).

General recommendations

Prior to initiating therapy with PRIACIN secondary causes of hypercholesterolemia (e.g. poorly controlled diabetes mellitus, hypothyroidism, nephrotic syndrome, dysproteinamias, obstructive liver disease, other drug therapy, alcoholism) should be identified and treated.

4.2. Posology and Method of Administration

The dosage range for PRIACIN is 5-80mg/day, given as a single dose in the evening. Adjustments of dosage, if required, should be made at intervals of not less than 4 weeks to a maximum of 80mg/day given as a single dose in the evening.

The 80mg dose is only recommended in patients at high risk for cardiovascular complications who have not achieved treatment goals on lower doses and when the benefits are expected to outweigh the potential risks.

Reductions at high risk of coronary heart disease (CHD) or with existing CHD

The usual starting dose of PRIACIN is 40mg/day given as a single dose in the evening in patients at high risk of CHD (with or without hyperlipidemia), i.e. patients with diabetes, history of stroke or other cerebrovascular disease, peripheral vessel disease, or with existing CHD. Drug therapy can be simultaneously initiated with a standard cholesterol-lowering diet and exercise.

Patients with Hypercholesterolemia (who are not in the risk categories above)

The patients should be placed on a standard cholesterol-lowering diet before receiving PRIACIN and should continue on this diet during the treatment with PRIACIN.

The usual starting dose is 20mg/day given as a single dose in the evening. Patients who require a large reduction in LDL-C (more than 45%) may be started at 40mg/day given as single dose in the evening. Patients with mild to moderate hypercholesterolemia can be treated with a starting dose of 10mg of PRIACIN. Adjustment of dosage, if required, should be made as specified above.

Homozygous familial hypercholesterolaemia

Based on the results of a controlled clinical study, the recommended dosage is 40 mg/day in the evening or 80 mg/day in 3 divided doses of 20 mg, 20 mg, and an evening dose of 40 mg. PRIACIN should be used as an adjunct to other lipid-lowering treatments (e.g., LDL apheresis) in these patients or if such treatments are unavailable

Concomitant therapy

PRIACIN is effective alone or in combination with bile acid sequestrants.

In patients taking fibrates (other than gemfibrozil and fenofibrate) concomitantly with PRIACIN, the dose of PRIACIN should not exceed 10 mg/day.

In patients taking amiodarone, verapamil or diltiazem concomitantly with PRIACIN, the dose of PRIACIN should not exceed 20mg/day.

In patients taking amlodipine or lipid-lowering dose of niacin ($\geq 1\text{g/day}$) concomitantly with PRIACIN, the dose of PRIACIN should not exceed 40mg/day.

Renal insufficiency

Because simvastatin does not undergo significant renal excretion, modification of dosage should not be necessary in patients with moderate renal insufficiency. In patients with severe renal insufficiency (creatinine clearance < 30 ml/min) dosages above 10 mg/ day should be carefully considered and if deemed necessary, implemented cautiously.

Dosage in paediatric patients (10-17 years of age) with heterozygous familial hypercholesterolemia

The recommended usual starting dose is 10mg once a day in the evening. The recommended dosing range is 10-40mg/day; the maximum recommended dose is 40mg/day. Doses should be individualized according to the recommended goal of therapy.

4.3. Contra-indications

The administration of simvastatin is contraindicated:

- for patients with hypersensitivity to simvastatin or to any of the other components of the tablet.
- for patients with active liver disease or unexplained persistent elevations of serum transaminases.
- during pregnancy and lactation.
- during concomitant administration of potent CYP3A4 inhibitors (e.g. itraconazole, ketoconazole, posaconazole, voriconazole, HIV protease inhibitors, boceprevir, telaprevir, erythromycin, clarithromycin, telithromycin and nefazodone) (see section 4.5)
- during concomitant administration of gemfibrozil, cycloporin or danazol

4.4. Special Warnings and Precautions for Use

Hepatic effects:

Persistent increases in serum transaminases may occur but when simvastatin is interrupted or discontinued the transaminase levels usually fall slowly to pre-treatment levels. It is recommended that liver function tests be performed before treatment begins and thereafter when clinically indicated. Patients titrated to the 80mg dose should receive an additional test prior to titration, 3 months after titration to the 80mg dose and periodically thereafter (e.g. semi-annually) for the first year of treatment. Special attention should be paid to patients who develop elevated serum transaminase levels and in these patients, measurements should be repeated promptly and then performed more frequently. If the transaminase levels show evidence of progression, particularly if they rise to 3 x ULN and are persistent, simvastatin should be discontinued. The product should be used with caution in patients who consume substantial quantities of alcohol.

As with other lipid lowering agents moderate (3 x ULN) elevations of serum transaminases have been reported following therapy with simvastatin. These changes appeared soon after initiation of therapy with simvastatin, were often transient, were not accompanied by any symptoms and interruption of treatment was not required.

Myopathy/Rhabdomyolysis:

Simvastatin, like other inhibitors of HMG-CoA reductase, occasionally causes myopathy manifested as muscle pain, tenderness or weakness with creatine kinase (CK) above ten times the upper limit of normal (ULN). Myopathy

sometimes takes the form of rhabdomyolysis with or without acute renal failure secondary to myoglobinuria, and very rare fatalities have occurred. The risk of myopathy is increased by high levels of HMG CoA reductase inhibitory activity in plasma. As with other HMG-CoA reductase inhibitors the risk of myopathy/rhabdomyolysis is dose related.

Creatine Kinase measurement

Creatine Kinase (CK) levels should not be measured following strenuous exercise or in the presence of any plausible alternative cause of CK increase as this makes value interpretation difficult. If CK levels are significantly elevated at baseline ($> 5 \times \text{ULN}$), levels should be re-measured within 5 to 7 days later to confirm the results.

Before the treatment:

All patients starting therapy with simvastatin, or whose dose of simvastatin is being increased, should be advised of the risk of myopathy and told to report promptly any unexplained muscle pain, tenderness or weakness.

Caution should be exercised in patients with pre-disposing factors for rhabdomyolysis. In order to establish a reference baseline value, a CK level should be measured before starting a treatment in the following situations:

- Elderly (age > 70 years)
- Renal impairment
- Uncontrolled hypothyroidism
- Personal or familial history of hereditary muscular disorders
- Previous history of muscular toxicity with a statin or fibrate
- Alcohol abuse

In such situations, the risk of treatment should be considered in relation to possible benefit and clinical monitoring is recommended. If a patient has previously experienced a muscle disorder on a fibrate or a statin treatment with a different member of the class should only be initiated with caution. If CK levels are significantly elevated at baseline ($> 5 \times \text{ULN}$), treatment should not be started.

Whilst on treatment

If muscular pain, weakness or cramps occur whilst a patient is receiving treatment with a statin, their CK levels should be measured. If these levels are found, in the absence of strenuous exercise, to be significantly elevated ($> 5 \times \text{ULN}$), treatment should be stopped. If muscular symptoms are severe and cause daily discomfort, even if CK levels are $< 5 \times \text{ULN}$, treatment discontinuation may be considered. If myopathy is suspected for any other reason, treatment should be discontinued.

If symptoms resolve and CK levels return to normal then re-introduction of the statin or introduction of an alternative statin may be considered at the lowest dose and with close monitoring.

Therapy with simvastatin should be temporarily stopped a few days prior to elective major surgery and when any major medical or surgical condition supervenes.

Measures to reduce the risk of myopathy caused by medicinal product interactions (see also section 4.5)

The risk of myopathy and rhabdomyolysis is significantly increased by concomitant use of simvastatin with potent inhibitors of CYP3A4 (such as itraconazole, ketoconazole, erythromycin, clarithromycin, telithromycin, HIV protease inhibitors, nefazodone), as well as gemfibrozil, cyclosporin and danazol (see section 4.2).

The risk of myopathy and rhabdomyolysis is also increased by concomitant use of other fibrates, lipid-lowering doses ($\geq 1\text{g/day}$) of niacin or by concomitant use of amiodarone or verapamil with higher doses of simvastatin (see sections 4.2 and 4.5). There is also a slight increase in risk when diltiazem is used with simvastatin 80 mg. Consequently, regarding CYP3A4 inhibitors, the use of simvastatin concomitantly with itraconazole, ketoconazole, HIV protease inhibitors, erythromycin, clarithromycin, telithromycin and nefazodone is contraindicated (see sections 4.3 and 4.5). If treatment with itraconazole, ketoconazole, erythromycin, clarithromycin or telithromycin is unavoidable, therapy with simvastatin must be suspended during the course of treatment. Moreover, caution should be exercised when combining simvastatin with certain other less potent CYP3A4 inhibitors: cyclosporin, verapamil, diltiazem (see sections 4.2 and 4.5). Concomitant intake of grapefruit juice and simvastatin should be avoided.

The dose of simvastatin should not exceed 10 mg daily in patients receiving concomitant medication with cyclosporin, danazol, gemfibrozil, or lipid-lowering doses ($\geq 1\text{g/day}$) of niacin. The combined use of simvastatin with gemfibrozil should be avoided, unless the benefits are likely to outweigh the increased risks of this drug combination. The benefits of the combined use of simvastatin 10 mg daily with other fibrates (except fenofibrate), niacin, cyclosporin or danazol should be carefully weighed against the potential risks of these combinations. (See sections 4.2 and 4.5.) Caution should be used when prescribing fenofibrate with simvastatin, as either agent can cause myopathy when given alone. The combined use of simvastatin at doses higher than 20 mg daily with amiodarone or verapamil should be avoided unless the clinical benefit is likely to outweigh the increased risk of

myopathy (see sections 4.2 and 4.5).

There have been very rare reports of an immune-mediated necrotizing myopathy (IMNM) during or after treatment with some statins. IMNM is clinically characterised by:

- persistent proximal muscle weakness and elevated serum creatine kinase, which persist despite discontinuation of statin treatment;
- muscle biopsy showing necrotizing myopathy without significant inflammation
- improvement with immunosuppressive agents

Myasthenia Gravis/Ocular Myasthenia

In few cases, statins have been reported to induce de novo or aggravate pre-existing myasthenia gravis or ocular myasthenia. PRIACIN should be discontinued in case of aggravation of symptoms. Recurrences when the same or a different statin was (re-) administered have been reported.

Excipient

Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

4.5. Interactions with other Medicaments and other forms of interaction

Pharmacodynamic interactions

Interactions with lipid-lowering medicinal products that can cause myopathy when given alone.

The risk of myopathy, including rhabdomyolysis, is increased during concomitant administration with fibrates and niacin (nicotinic acid) ($\geq 1\text{g/day}$). Additionally, there is a pharmacokinetic interaction with gemfibrozil resulting in increased simvastatin plasma levels (see below Pharmacokinetic Interactions and Sections 4.2 and 4.4). When simvastatin and fenofibrate are given concomitantly, there is no evidence that the risk of myopathy exceeds the sum of the individual risks of each agent. Adequate pharmacovigilance and pharmacokinetic data are not available for other fibrates.

Pharmacokinetic interactions

Prescribing recommendations for interacting agents are summarised in the table below (further details are provided in the text: see also sections 4.2, 4.3 and 4.4).

Drug interactions Associated with Increased Risk of Myopathy/Rhabdomyolysis

Interacting agents	Prescribing recommendations
Potent CYP 3A4 inhibitors: Itracomazole, Ketoconazole, Posaconazole, Voriconazole, Erythromycin, Clarithromycin, Telithromycin, HIV protease inhibitors, Boceprevir, Telaprevir, Nefazodone	Contraindicated with simvastatin
Cyclosporine Danazol	Contraindicated with simvastatin
Other fibrates (other than gemfibrozil and fenofibrate)	Do not exceed 10mg simvastatin daily
Niacin ($\geq 1\text{g/day}$)	Do not exceed 40mg simvastatin daily
Amiodarone Verapamil and Diltiazem	Do not exceed 20mg simvastatin daily
Grapefruit juice	Avoid grapefruit juice when taking simvastatin

Effects of other medicinal products on simvastatin

Interactions involving CYP3A4

Potent inhibitors of CYP3A4: Simvastatin is a substrate of cytochrome P450 3A4. Potent inhibitors of cytochrome P450 3A4 increase the risk of myopathy and rhabdomyolysis by increasing the concentration of HMG CoA reductase inhibitory activity in plasma during simvastatin therapy. Such inhibitors include itraconazole, ketoconazole, posaconazole, voriconazole, erythromycin, clarithromycin, telithromycin, HIV protease inhibitors, boceprevir, telaprevir and nefazodone.

Concomitant administration of itraconazole resulted in a more than 10 fold increase in exposure to simvastatin acid (the active beta hydroxyacid metabolite). Telithromycin caused an 11 fold increase in exposure to simvastatin acid.

Concomitant use with medicines labelled as having a potent inhibitory effect on CYP3A4 at therapeutic doses (e.g.: itraconazole, ketoconazole, posaconazole, voriconazole, erythromycin, clarithromycin, telithromycin, HIV protease inhibitors, boceprevir, telaprevir or nefazodone) is contraindicated. If treatment with potent CYP3A4 inhibitors is unavoidable, therapy with simvastatin should be suspended during the course of treatment.

Caution should be exercised when combining simvastatin with certain other less potent CYP3A4 inhibitors: cyclosporine, verapamil, diltiazem (see sections 4.2 and 4.4).

Gemfibrozil, cyclosporine or danazol

Concomitant use of these drugs with simvastatin is contraindicated.

Other Drugs

- Other fibrates: The dose of simvastatin should not exceed 10mg daily in patients receiving concomitant medication with fibrates other than gemfibrozil or fenofibrate. When simvastatin and fenofibrate are given concomitantly, there is no evidence that the risk of myopathy exceeds the sum of the individual risks of each agent. Caution should be used when prescribing fenofibrate with simvastatin, as either agent can cause myopathy when given alone. Addition of fibrates to simvastatin typically provides little additional reduction in LDL-C, but further reductions of TG and further increases in HDL-C may be obtained. Combinations of fibrates with simvastatin have been used without myopathy in small short-term clinical studies with careful monitoring. **Concurrent use of fibrates may cause severe myositis and myoglobinuria**
- Amiodarone: In a clinical trial, myopathy was reported in 6% of patients receiving simvastatin 80mg and amiodarone. The dose of simvastatin should not exceed 20mg daily in patients receiving concomitant medication with amiodarone.
- Calcium channel blockers:
 - Verapamil or diltiazem: In a clinical trial, patients on diltiazem treated concomitantly with simvastatin 80mg had an increased risk of myopathy. The dose of simvastatin should not exceed 20mg daily in patients receiving concomitant medication with verapamil or diltiazem
 - Amlodipine: In a clinical trial, patients on amlodipine treated concomitantly with simvastatin 80mg had a slightly increased risk of myopathy. The dose of simvastatin should not exceed 40mg daily in patients receiving concomitant medication with amlodipine
 - Niacin ($\geq 1\text{g}$): the dose of simvastatin should not exceed 40mg daily in patients receiving concomitant medication with niacin (nicotinic acid) $\geq 1\text{g}$. Cases of myopathy/rhabdomyolysis have been observed with simvastatin co-administered with lipid-modifying doses ($\geq 1\text{g}$) of niacin

Grapefruit juice

Grapefruit juice inhibits cytochrome P450 3A4. Concomitant intake of large quantities (over 1 litre daily) of grapefruit juice and simvastatin increases in exposure to simvastatin acid. Intake of grapefruit juice during treatment with simvastatin should therefore be avoided.

Effects of simvastatin on pharmacokinetics of other medicinal products

Simvastatin does not have an inhibitory effect on cytochrome P450 3A4. Therefore, simvastatin is not expected to affect plasma concentrations of substances metabolised via cytochrome P450 3A4.

Oral anticoagulants

Simvastatin 20-40 mg/day modestly potentiates the effect of coumarin anticoagulants. Very rare cases of elevated INR have been reported. In patients taking coumarin anticoagulants, prothrombin time should be determined before starting simvastatin and frequently enough during early therapy to ensure that no significant alteration of prothrombin time occurs. Once a stable prothrombin time has been documented, prothrombin times can be monitored at the intervals usually recommended for patients on coumarin anticoagulants. If the dose of simvastatin is changed or discontinued, the same procedure should be repeated. Simvastatin therapy has not been associated with bleeding or with changes in prothrombin time in patients not taking anticoagulants.

4.6. Pregnancy and Lactation

Pregnancy

PRIACIN is contraindicated during pregnancy (see section 4.3).

Safety in pregnant women has not been established. No controlled clinical trials with simvastatin have been conducted in pregnant women. Rare reports of congenital anomalies following intrauterine exposure to HMG-CoA reductase inhibitors have been received. However, in an analysis of approximately 200 prospectively followed pregnancies exposed during the first trimester to PRIACIN or another closely related HMG CoA reductase inhibitor, the incidence of congenital anomalies was comparable to that seen in the general population. This number of pregnancies was statistically sufficient to exclude a 2.5 fold or greater increase in congenital anomalies over the background incidence.

Although there is no evidence that the incidence of congenital anomalies in offspring of patients taking PRIACIN or another closely related HMG CoA reductase inhibitor differs from that observed in the general population, maternal treatment with PRIACIN may reduce the foetal levels of mevalonate which is a precursor of cholesterol biosynthesis. Atherosclerosis is a chronic process, and ordinarily discontinuation of lipid-lowering medicinal products during pregnancy should have little impact on the long-term risk associated with primary hypercholesterolaemia. For these reasons, PRIACIN should not be used in women who are pregnant, trying to become pregnant or suspect they are

pregnant.

Treatment with PRIACIN should be suspended for the duration of pregnancy or until it has been determined that the woman is not pregnant. (See section 4.3.)

Lactation

It is not known whether simvastatin or its metabolites are excreted in human milk. Because many medicinal products are excreted in human milk and because of the potential for serious adverse reactions, women taking PRIACIN should not breast-feed their infants (see section 4.3).

4.7. Effects on Ability to Drive and Use Machines

PRIACIN has no or negligible influence on the ability to drive and use machines. However, when driving vehicles or operating machines, it should be taken into account that dizziness has been reported rarely in post-marketing experiences.

4.8. Undesirable Effects

The following adverse events have been reported during clinical studies and/or post-marketing use.

Blood and lymphatic system disorders:

Rare: anaemia

Nervous system disorders:

Rare: headache, paresthesia, dizziness, peripheral neuropathy

Frequency 'not known': myasthenia gravis

Gastrointestinal disorders:

Rare: constipation, abdominal pain, flatulence, dyspepsia, diarrhoea, nausea, vomiting, pancreatitis

Hepato-biliary disorders:

Rare: hepatitis/jaundice

Skin and subcutaneous tissue disorders:

Rare: rash, pruritus, alopecia

Musculoskeletal, connective tissue and bone disorders:

Rare: myopathy, rhabdomyolysis (see section 4.4), myalgia, muscle cramps

Not known: Immune-mediated necrotizing myopathy

Eye disorder:

Frequency 'not known': ocular myasthenia

General disorders and administration site conditions:

Rare: asthenia

An apparent hypersensitivity syndrome has been reported rarely which has included some of the following features: angioedema, lupus-like syndrome, polymyalgia rheumatica, dermatomyositis, vasculitis, thrombocytopenia, eosinophilia, ESR increased, arthritis and arthralgia, urticaria, photosensitivity, fever, flushing, dyspnoea and malaise.

Investigations:

Rare: increases in serum transaminases (alanine, aminotransferase, aspartate aminotransferase, γglutamyl transpeptidase) (see section 4.4 Hepatic effects), elevated alkaline phosphatase; increase in CK serum levels (see section 4.4).

There have been rare post-marketing reports of cognitive impairment (e.g. memory loss, forgetfulness, amnesia, memory impairment, confusion) associated with statin use. These cognitive issues have been reported for all statins. The reports are generally non-serious and reversible upon statin discontinuation, with variable times to symptom onset (1 day to years) and symptom resolution (median 3 weeks)

Increase in HbA1c and fasting blood glucose have been reported with statins. The risk of hyperglycemia, however, is outweighed by the reduction in vascular risk with statins.

4.9. Overdose

To date, a few cases of overdose have been reported; the maximum dose taken was 3.6 g. All patients recovered without sequelae. There is no specific treatment in the event of overdose. In this case, symptomatic and supportive

measures should be adopted.

5. Pharmacological Properties

5.1. Pharmacodynamic Properties

Pharmacotherapeutic group: HMG CoA reductase inhibitor

ATC-Code: C10A A01

After oral ingestion, simvastatin, which is an inactive lactone, is hydrolyzed in the liver to the corresponding active beta hydroxyacid form which has a potent activity in inhibiting HMG CoA reductase (3 hydroxy – 3 methylglutaryl CoA reductase). This enzyme catalyses the conversion of HMG CoA to mevalonate, an early and rate-limiting step in the biosynthesis of cholesterol. Simvastatin has been shown to reduce both normal and elevated LDL C concentrations. LDL is formed from very-low-density protein (VLDL) and is catabolised predominantly by the high affinity LDL receptor. The mechanism of the LDL lowering effect of Simvastatin may involve both reduction of VLDL-cholesterol (VLDL C) concentration and induction of the LDL receptor, leading to reduced production and increased catabolism of LDL C. Apolipoprotein B also falls substantially during treatment with Simvastatin. In addition, Simvastatin moderately increases HDL C and reduces plasma TG. As a result of these changes the ratios of total-to HDL C and LDL-to HDL C are reduced.

5.2. Pharmacokinetic Properties

Simvastatin is an inactive lactone which is readily hydrolyzed in vivo to the corresponding beta hydroxyacid, a potent inhibitor of HMG CoA reductase. Hydrolysis takes place mainly in the liver; the rate of hydrolysis in human plasma is very slow.

Absorption

In man simvastatin is well absorbed and undergoes extensive hepatic first-pass extraction. The extraction in the liver is dependent on the hepatic blood flow. The liver is the primary site of action of the active form. The availability of the beta hydroxyacid to the systemic circulation following an oral dose of simvastatin is less than 5 % of the dose. Maximum plasma concentration of active inhibitors is reached approximately 1 2 hours after administration of simvastatin. Concomitant food intake does not affect the absorption.

There is no accumulation of medicinal product after multiple dosing.

Distribution

Both simvastatin and its β -hydroxy acid metabolite are highly bound (>95%) to human plasma proteins.

Elimination

Simvastatin is a substrate of CYP3A4 (see sections 4.3 and 4.5). The major metabolites of simvastatin present in human plasma are the beta hydroxyacid and four additional active metabolites. Following an oral dose of radioactive simvastatin to man, 13 % of the radioactivity is excreted in the urine and 60 % in the faeces within 96 hours. The amount recovered in the faeces represents absorbed medicinal product equivalents excreted in bile as well as unabsorbed medicinal product. After intravenous injection of the beta hydroxyacid metabolite, the half-life is about 1.9 hours. An average of only 0.3 % of the IV dose is excreted in urine as inhibitors.

6. Pharmaceutical Particulars

6.1. List of Excipients

PRIACIN 20mg and 40mg tablets also contain: Core: Lactose monohydrate, Pregelatinised starch, microcrystalline cellulose, Butylated hydroxyanisole, Ascorbic acid, Citric acid monohydrate, Magnesium stearate, Purified water
Coating: Hydroxypropylmethyl cellulose, Hydroxypropyl cellulose, Titanium dioxide (E171), Talc and red iron oxide (E172). PRIACIN 20mg also contain yellow iron oxide (E172).

6.2. Incompatibilities None known

6.3. Shelf Life

2 years after the production date.

6.4. Special Precautions for Storage

Store below 30°C in the original package.

6.5. Nature and Contents of Container

PRIACIN tablets 20mg and 40mg are packed in polyvinylchloride /polyethylene/polyvinylidene chloride – aluminium (PVC/PE/PVdC-Al) blister. The number of tablets in one blister is 10. Boxes containing 30, 50 & 100 tablets are available.

6.6. Instruction for Use/Handling

None

7. Product Registration Holder

Komedic Sdn Bhd, No.4 Jalan PJS11/14, Bandar Sunway, 46150 Petaling Jaya

8. Manufacturer

Medochemie Ltd, (Central Factory), Konstantinoupoleos 1-10, Limassol, 3011, Cyprus

9. Date of (Partial) Revision of the Text:

September 2025